INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pembrolizumab + cisplatin + fluorouracil

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00706
Version 3

Every 3 weeks; up to 6 combination cycles, then pembrolizumab alone.

Treatment stages

01 **Combination treatment**
Up to 6 cycles · every 3 weeks

02 **Pembrolizumab alone**
Every 3 weeks · ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call the oncology team promptly for fever, sudden illness, diarrhoea, painful mouth, reduced drinking, new breathlessness or immune symptoms. Chest pain during or after fluorouracil needs urgent assessment. Severe chest pain, breathing difficulty, collapse or a seizure is an emergency. Early severe fluorouracil symptoms can occur despite dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

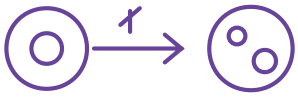
How each part of treatment works

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

PD-1 CHECKPOINT IMMUNOTHERAPY

Pembrolizumab

Keytruda



Releases an immune brake

Pembrolizumab blocks PD-1, a signal that can limit the activity of immune cells. This can help them attack cancer, but can also cause inflammation in healthy organs during treatment or after it ends.

PD-1 blocked → Immune cells more active → Healthy organs can inflame

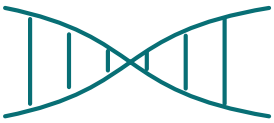
What to watch for

Report new diarrhoea, cough, breathlessness, rash, headache, unusual weakness, thirst or changes in urine promptly.

PLATINUM CHEMOTHERAPY

Cisplatin

Cisplatin



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Cisplatin forms links in DNA that make it harder for cells to repair and divide. In this regimen it is combined with fluorouracil and pembrolizumab to treat cancer throughout the body. Healthy cells can be affected, which is why kidney, blood, hearing and nerve checks matter.

DNA damage → Cell repair disrupted → Cancer growth reduced

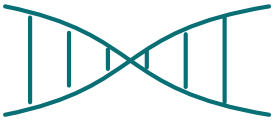
What to watch for

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

ANTIMETABOLITE CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts production of DNA and RNA

Fluorouracil interferes with the materials cells need to make DNA and RNA. A pump gives it over the prescribed days. DPD is an enzyme that helps break it down; testing before treatment reduces risk but cannot prevent every severe reaction.

Cell building blocks disrupted → Cells cannot divide normally → Pump provides prescribed exposure

What to watch for

Report severe diarrhoea, mouth sores, chest pain, sudden confusion, sore palms or soles, or unexpectedly early severe symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep your pathology and agreed treatment goal with your records.

Site, type and stage

Ask which part of the head or neck the cancer started in, what the biopsy shows and how far it has spread. Site, stage, previous treatment and your health affect the options. HPV/p16 testing is relevant to some sites; it is not the same test as PD-L1.

Which pathway applies?

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma in adults with tumour PD-L1 CPS ≥1. This guide should match the named treatment on your prescription.

What tumour tests mean

PD-L1 CPS is used to select this pembrolizumab combination; DPD testing assesses fluorouracil safety. Neither is a general blood tumour marker that measures response.

Understanding the evidence

KEYNOTE-048 studied first-line treatment for recurrent or metastatic squamous head and neck cancer.

The original 2019 report supported pembrolizumab with platinum and fluorouracil in this setting. The chemotherapy group included cisplatin or carboplatin; it does not provide a separate randomised survival estimate for this specific platinum choice.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment stages

Treatment stages

01 **Combination treatment**
Up to 6 cycles · every 3 weeks

02 **Pembrolizumab alone**
Every 3 weeks · ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

UP TO 6 CYCLES · EVERY 3 WEEKS

Combination treatment

1 2 3 4 5 6

Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time. The planned combination limit is six cycles; toxicity may change the plan earlier.

EVERY 3 WEEKS · ONGOING REVIEW

Pembrolizumab alone

Ongoing maintenance

Pembrolizumab 200 mg over 30 minutes every 21 days continues after the combination phase when appropriate, until progression or unacceptable toxicity under this regimen. A six-cycle chemotherapy record is not a six-cycle total course; the protocol does not impose an automatic 24-month stop.

Dates and treatment decisions come from your team. A delayed or omitted dose does not count as treatment received.

BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Source-required blood tests are distinguished from tests ordered for a particular symptom or disease plan. Imaging is not interchangeable with blood safety tests.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00706 v3: official regimen

Tests in your plan

Full blood count

WHEN
Before every cycle

Checks cells that fight infection, carry oxygen and help stop bleeding.

Kidney and liver tests

WHEN
Before every cycle

Support safe dosing and detect organ problems.

Thyroid tests

WHEN
Every 3–6 weeks during pembrolizumab; sooner if indicated.

Can detect immune-related hormone changes before or with symptoms.

THE WHOLE TREATMENT JOURNEY

Pembrolizumab + cisplatin + fluorouracil: treatment and checks

Every 21 days: pembrolizumab and cisplatin on Day 1; fluorouracil over Days 1–4. Up to 6 combination cycles, then pembrolizumab alone while appropriate. Every dose depends on safety and benefit review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests NCCP	Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/core antibody and hepatitis C; tumour PD-L1 CPS and DPD assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves.	Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Treatment and appointments NCCP	Confirm the exact diagnosis, intent, prescription and calendar.	Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.	Use the prescribed support plan and contact route.	Pembrolizumab 200 mg over 30 minutes every 21 days continues after the combination phase when appropriate, until progression or unacceptable toxicity under this regimen. A six-cycle chemotherapy record is not a six-cycle total course; the protocol does not impose an automatic 24-month stop.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing NCCP	Agree oral care and assess weight, intake and swallowing. If radiation is planned separately, arrange its dental preparation with the team.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.
Imaging and tumour assessment NCCP	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

What each check is for

1 **Combination treatment**

Up to 6 cycles · every 3 weeks

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

2 **Pembrolizumab alone**

Every 3 weeks · ongoing review

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Agree the access device

The team discusses a port or peripherally inserted central catheter (PICC) and trains you in its care.

2 · Before Day 1

Confirm the actual pump plan

Four daily 22-hour infusions or two 48-hour infusions may be prescribed. Confirm all change and disconnection appointments.

3 · Day 1

Trained staff connect fluorouracil

After the Day 1 infusions, staff start the prescribed pump. Do not calculate or alter its rate.

4 · Days 1–4

Follow the chosen infusion schedule

Daily pump changes or a change after the first 48-hour infusion are arranged by the unit. Protect the line and report leaks or unexpected progress.

5 · At prescribed completion

Professional disconnection

Timing is based on actual connection and infusion duration. Keep the appointment; do not disconnect or dispose of the pump yourself.

6 · Between cycles

Continue line care

The access device may stay in place. Follow its dressing, flushing and infection precautions, including during maintenance if it remains.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm the exact regimen, treatment aim, baseline tests, transport, support medicines and actual urgent contact numbers. Bring any hearing concerns, swallowing problems and medicine list.

STEP 2

Time in the unit

Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.

STEP 3

Pump visits and line care

Record every pump change and disconnection appointment. Device problems need the unit’s instructions; do not reconnect, adjust or remove the line yourself.

STEP 4

Practical help

Tell the team about travel, cost, work, caring responsibilities, communication or reading needs. Ask whether a support person or interpreter can attend and how treatment changes will be explained.

STEP 5

Leaving the unit

Check the next visit and blood-test dates, support medicines and who to call if unwell. Have the team replace example phone numbers before using the wallet card.

Recovery, assessment and longer-term care

Treatment completion and recovery are different milestones.

Continuing symptoms

Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.

Cancer assessment

Your team sets examinations and imaging around the disease, treatment and symptoms. Record who arranges each test and who explains the result. An unbooked scan is not automatically due at a fixed week because a general guide says so.

Information for other clinicians

Keep the regimen name, last treatment date, ongoing medicines and relevant alert card. Tell clinicians about earlier immune treatment or head and neck radiation even after the course has ended.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Combination cycle

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Pembrolizumab + Cisplatin	 **Day 1** Day 1 infusions after safety review.
Fluorouracil pump	 **Days 1–4** 4 × 22 hours or 2 × 48 hours; actual appointments depend on connection time.
Symptoms and recovery	 **Days 1–21** No guaranteed safe days; immune effects may also occur after treatment stops.

Treatment sequence: Every 21 days: pembrolizumab and cisplatin on Day 1; fluorouracil over Days 1–4. Up to 6 combination cycles, then pembrolizumab alone while appropriate. Every dose depends on safety and benefit review.

Appointments and administration: Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.

Baseline and ongoing checks: Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/core antibody and hepatitis C; tumour PD-L1 CPS and DPD assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves. Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.

Recovery and follow-up: Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.

> **Why these bands stay broad:** The days show a usual treatment pattern, not a prediction of when symptoms will happen. Your team confirms dates and any changes. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00706 v3: official regimen ↗

There are no guaranteed "safe days". Your prescription and team's instructions take priority. Contact the team about new symptoms rather than waiting for a scheduled visit.

Pembrolizumab maintenance cycle

Day 1 → Day 21: a general pattern

Treatment or check	Timing in this view
Pembrolizumab alone	1 — 21 **Day 1** 200 mg over 30 minutes every 21 days when appropriate.

Treatment sequence: Every 21 days: pembrolizumab and cisplatin on Day 1; fluorouracil over Days 1–4. Up to 6 combination cycles, then pembrolizumab alone while appropriate. Every dose depends on safety and benefit review.

Appointments and administration: Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.

Baseline and ongoing checks: Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/core antibody and hepatitis C; tumour PD-L1 CPS and DPD assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves. Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.

Recovery and follow-up: Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.

Why these bands stay broad: No platinum or fluorouracil pump is scheduled in this maintenance phase; immune monitoring continues. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00706 v3: official regimen ↗

Between-treatment notes

Day 1: Infusions and pump connection

Receive the Day 1 medicines after safety checks; trained staff start fluorouracil. Confirm the exact pump schedule, support medicines and contact plan.

Day 2–4: Fluorouracil continues

Four 22-hour infusions or two 48-hour infusions as prescribed; changes and completion are arranged by staff. Do not adjust the pump. Call for unexpected pump progress or symptoms.

Day 5–21: Recovery and review

No planned chemotherapy after completion of the pump in this reference cycle. Pembrolizumab immune effects can occur at any time. Report symptoms early and arrange the next required tests.

Support around the medicines

1 One written medicine plan

Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.

2 Immune effects and maintenance

Pembrolizumab can cause inflammation in healthy organs even after chemotherapy has finished. Keep the alert card and report new symptoms promptly. Steroids or hormone replacement may be needed for specific complications; do not start or stop them without the treating team.

3 Hydration and kidney protection

Cisplatin needs prescribed fluids before and after infusion. Follow the team's drinking advice and report much less urine, vomiting or inability to drink. Do not change fluid intake or diuretics without advice.

4 Food, fluids and weight

A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.

5 Oral and dental care

Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.

6 Swallowing and communication

A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.

7 Fertility, pregnancy and medicines

Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.

Treatment choices and your priorities

The consent conversation explains the boundaries of the plan.

Benefit and alternatives

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you. Ask how alternatives compare for your health, goals and previous treatment. A study result cannot predict an individual outcome.

A pause is a treatment decision

The clinician may delay, change or stop treatment because of symptoms, test results or your priorities. Record which component is affected and what is needed before restarting. Do not restart from an old prescription.

Combining treatment approaches

This regimen is systemic treatment. Surgery or radiotherapy may have a separate role for an individual symptom or disease plan; concurrent radiation is not automatically part of this combination.

Preparing for and receiving treatment

Plan the appointments and support around the prescribed schedule.

If an appointment changes

Contact the unit for revised dates after a missed or delayed visit. A missed appointment is not a completed dose. Ask whether blood tests, premedicines or pump appointments also need to change.

What the record shows

Write the actual treatment date, phase and any change. The wallet card is a brief record; the passport holds the fuller history. It does not replace the hospital treatment record.

Your pump and line

Ask staff to demonstrate the specific device, check progress and explain spill or emergency steps. Call for leakage, damage, disconnection, unexpected completion or a pump that seems not to run. Do not change settings, disconnect or reconnect it yourself.

Between visits: symptoms and medicines

Use the agreed home plan and contact pathway.

Symptoms have no fixed calendar

Some effects appear soon after treatment and others build over weeks. Immune effects may begin during maintenance or after the last pembrolizumab dose. Do not wait for the next appointment if a symptom is new, severe or worsening.

A medicine plan you can use

Keep prevention, rescue and regular medicines clearly separated on one written list. Record what you have taken and the response, so the team can adjust it. Do not double a missed dose unless your prescriber tells you to.

Mouth, teeth and jaw care

Dental preparation and gentle daily care reduce avoidable problems.

Before treatment

Arrange dental assessment and oral care with the team. If radiotherapy is planned separately, the dental and radiation teams decide any necessary treatment and healing time before it starts.

During treatment

Follow the prescribed brushing, fluoride, rinse and pain-relief advice. Report ulcers, infection, new bleeding, difficulty wearing dentures or worsening jaw opening. Avoid starting a new mouthwash without checking if the mouth is sore.

If you have had head and neck radiation

Earlier head and neck radiation can affect saliva, teeth and healing long afterwards. Tell any dentist about it before invasive treatment. If you have not had radiation, follow the oral-care advice appropriate to your current treatment.

Eating, drinking and maintaining strength

The aim is adequate intake and comfort during treatment.

Recognising an intake problem

Record weight and report food or drink becoming harder to manage. Pain, taste change, dry mouth or nausea can each need a different response; a dietitian can adapt the plan to you.

If ordinary food is not enough

The team may suggest food fortification, supplements or tube feeding when intake or swallowing safety requires it. A tube is a support option, not a judgement about how well you have tried to eat. Training and follow-up are needed.

Fluid needs are individual

Ask for a realistic drinking plan that accounts for kidney/heart problems and any feeding tube. Persistent vomiting, dizziness, markedly reduced urine or inability to drink needs urgent contact.

Diarrhoea needs the correct response

Fluorouracil and pembrolizumab can both cause diarrhoea through different mechanisms. Contact the team early rather than assuming it is ordinary chemotherapy diarrhoea; use rescue medicine only under the written plan.

Swallowing, voice and communication

A speech and language therapist can help protect function.



Swallowing assessment

Report coughing or choking with food or drink, a wet-sounding voice after swallowing, food sticking or repeated chest infections. The team may need to assess aspiration risk and safe textures.

Exercises and rehabilitation

Do swallowing or jaw exercises only as taught for your situation. Treatment and recovery can change what is comfortable and safe. Ask for reassessment when symptoms change.

Communication needs

Tell the team about changes in speech, voice or hearing. Ask for written instructions, communication aids, an interpreter or help involving a support person.

Daily life and emotional support

Practical adjustments can make the plan easier to follow.

Energy and activity

Balance rest with manageable activity according to your ability and team advice. Ask for help if weakness, falls, sleep difficulty or pain interferes with daily life.

Smoking and alcohol

Ask for support to stop smoking and discuss alcohol with the team. Mouth irritation, medicine interactions and head and neck recovery can change what is safe for you.

Emotional and practical concerns

Tell the team about anxiety, appearance or intimacy concerns, finances, travel and caring responsibilities. Ask which local supports are available; resources can be added to your record when confirmed.

Your records and additional help

Keep essential instructions available on paper.

The passport and wallet card

The three-page companion gives the essentials. Use the passport for the agreed plan, visits, test results, symptoms and questions. The wallet QR opens this exact treatment's further information; it contains no personal clinical record.

Contacts before patient use

The phone numbers printed as examples must be replaced and checked by the treating service. Add the unit's daytime, out-of-hours and backup route; keep this information where you and a support person can find it.

If you prefer paper

Use the full printable handbook in its chapter order and write questions in the passport. You do not need to scan the QR or follow an internet link to use the essential treatment and safety information.

Questions worth asking

Which tests are required?

Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/core antibody and hepatitis C; tumour PD-L1 CPS and DPD assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves. Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.

What if a treatment is held?

The team records the reason and recovery or tests needed before restarting. Do not count an omitted dose as completed or restart from an old plan.

Try explaining the plan back: Explain which treatment phase you are in, what happens next, which tests are due and which symptoms would make you call today. Ask the team to clarify anything you cannot explain.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination treatment

Active medicines: Pembrolizumab + Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
High-emetic-risk anti-sickness plan	Clinician/pharmacy to complete	Before platinum and for delayed symptoms as prescribed Use the current local/NCCP policy; reconcile prevention and rescue medicines into one schedule.
Cisplatin hydration and electrolyte plan	Clinician/pharmacy to complete	Before and after cisplatin; home fluids individualised Check kidney function, fluid balance and relevant comorbidities; do not issue an automated fluid prescription.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Pembrolizumab can cause inflammation in healthy organs even after chemotherapy has finished. Keep the alert card and report new symptoms promptly. Steroids or hormone replacement may be needed for specific complications; do not start or stop them without the treating team.
- Cisplatin needs prescribed fluids before and after infusion. Follow the team's drinking advice and report much less urine, vomiting or inability to drink. Do not change fluid intake or diuretics without advice.
- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- Fluorouracil is low emetic risk but the combination follows the high-risk platinum plan. The pump schedule and any authorised diarrhoea rescue plan must be explicit.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Pembrolizumab alone

Active medicines: Pembrolizumab

Emetogenic risk: minimal

Incomplete clinician/pharmacy review draft. Individualise and authorise before use.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Pembrolizumab can cause inflammation in healthy organs even after chemotherapy has finished. Keep the alert card and report new symptoms promptly. Steroids or hormone replacement may be needed for specific complications; do not start or stop them without the treating team.

- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- No routine prophylactic antiemetic is specified for pembrolizumab alone. Do not carry platinum antiemetics or hydration forward automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / hormones / eyes

Pembrolizumab

Report severe headache, visual change, unusual weakness, thirst, dizziness or frequent urination.

3 Lungs / breathing

Pembrolizumab

New cough or breathlessness needs prompt assessment; severe breathing difficulty is an emergency.

5 Liver

Pembrolizumab + chemotherapy

Blood tests may show inflammation first. Report yellow skin or eyes, dark urine or unusual itching.

7 Bowels / abdomen

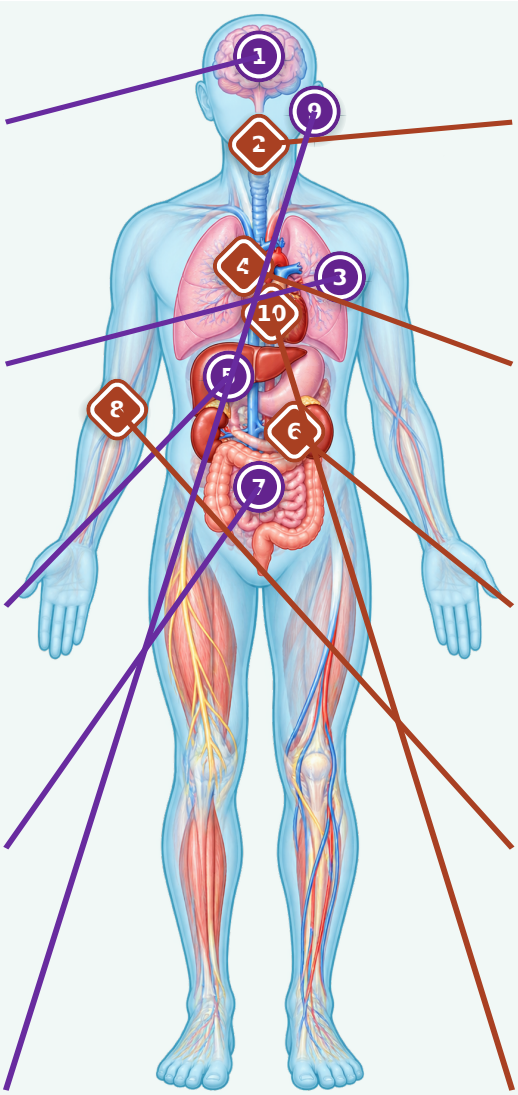
Fluorouracil + pembrolizumab

Call early for diarrhoea, blood or mucus, pain or dehydration. Different causes need different treatment.

9 Hearing / nerves

Cisplatin

Report tinnitus, hearing change, tingling, weakness or balance problems before the next dose.



2 Mouth / throat

Fluorouracil

Report ulcers, painful swallowing or difficulty drinking; early severe symptoms need urgent review.

4 Heart / chest

Fluorouracil + pembrolizumab

Chest pain, palpitations, collapse or marked muscle weakness needs urgent assessment.

6 Kidneys / fluids

Cisplatin + pembrolizumab

Report reduced urine, swelling, persistent vomiting or difficulty drinking; kidney results also guide platinum treatment.

8 Skin / hands / feet

Fluorouracil + pembrolizumab

Report rash, painful palms or soles. Blistering, peeling or rash with fever needs urgent assessment.

10 Blood cells / infection

Chemotherapy

Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call the oncology team promptly for fever, sudden illness, diarrhoea, painful mouth, reduced drinking, new breathlessness or immune symptoms. Chest pain during or after fluorouracil needs urgent assessment. Severe chest pain, breathing difficulty, collapse or a seizure is an emergency. Early severe fluorouracil symptoms can occur despite DPD testing.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00706 v3 · pSee regimen treatment, tests and supportive-care sections**
   https://healthservice.hse.ie/documents/6880/706_V3_Pembrolizumab_CISplatin_5FU_.pdf
2. **eviQ: port**
   https://www.eviq.org.au/p/507
3. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
4. **eviQ: pump**
   https://www.eviq.org.au/p/502
5. **NCCP head and neck regimen catalogue**
   https://healthservice.hse.ie/staff/information-healthcare-workers/nccp/head-and-neck-sact-regimens/
6. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
7. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
8. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
9. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
10. **Irish Cancer Society: head and neck radiotherapy effects**
    https://www.cancer.ie/cancer-treatments/radiotherapy/side-effects-from-radiotherapy-to-the-head-and-neck-area
11. **EMA: Keytruda product information and patient leaflet**
    https://www.ema.europa.eu/en/documents/product-information/keytruda-epar-product-information_en.pdf
12. **KEYNOTE-048: original 2019 randomised trial report**
    https://pubmed.ncbi.nlm.nih.gov/31679945/
13. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf